

The Constraint

Why the current way of working cannot reach the board's target

MedFlex today vs. what the board requires

- Now: **\$14M revenue, 8 coordinators**, 4.2 hours average time to fill a shift
- Board target: **\$200M in 24 months** — about 14 times more business
- At today's productivity: **112 coordinators would be needed** to handle that volume manually

The problem is not effort — it is how the work is structured.

To reach the board's target, coordinators must stop doing every match themselves and start supervising agents that handle standard cases automatically. This is a design problem, not a staffing problem.

What Is Breaking Today

Three problems that get worse as volume grows

Process problems — not coordinator mistakes

- **Too slow** — Hospitals send requests to several agencies at the same time. At 4.2 hours to respond, MedFlex loses contracts before a proposal is even sent.
- **Hard ceiling on capacity** — 8 coordinators doing 120 decisions per day is the maximum. When more requests come in, a queue builds up. The queue makes the delay worse.
- **Knowledge locked in people** — Senior coordinators know which nurse fits which hospital. This knowledge is not written down. New people take longer and get inconsistent results. When someone leaves, the knowledge is gone.

Problems found during discovery

- **Manual email parsing** — Every hospital email is read and interpreted by hand before matching starts. This is a hidden delay step.
- **12% no-show rate** — Nurses receive an SMS and silence is treated as acceptance. MedFlex finds out about a no-show when the hospital calls.
- **Double-booking risk** — The same nurse can be offered to two hospitals at once. There is no automatic way to resolve conflicts when this happens at scale.

The Solution

Three agents — existing hospital and nurse channels stay the same

Agent relay: read → match → confirm

- **Agent 1 — Reader**

Converts unstructured hospital emails into structured shift records. Coordinator gets a 5-minute preview before the record moves forward — catches any misreads before they affect matching.

- **Agent 2 — Matchmaker**

Scores each candidate on credentials, availability, distance, and hospital history. High-confidence matches (above 85%) are sent to the hospital automatically. Mid-confidence matches are sent with a 90-minute coordinator recall window. Low-confidence cases go to the coordinator with a ranked list of options.

- **Agent 3 — Follow-up**

Asks the nurse to confirm explicitly after the hospital accepts. Escalates if there is no reply. Starts an emergency re-match if a no-show is reported.

Coordinator role: managing exceptions, not doing routine matching.

- **What stays the same:** ServiceNow queue, hospital email and portal channels, nurse SMS and email

How Risk Is Managed

Trust is built step by step — not by switching everything at once

Four safeguards in the design

- **Step-by-step rollout** — Phase 1 covers only the highest-confidence 50% of daily volume. Coordinators review all other cases. Full automation is earned through results, not assumed.
- **90-minute coordinator recall window** — Every mid-confidence proposal can be cancelled within 90 minutes. If the coordinator does not review it in time, the team lead is notified. A proposal is never automatically approved without a human seeing it.
- **Nurse reservation** — When a nurse is proposed to a hospital, a 15-minute soft lock prevents the same nurse from being offered to another hospital at the same time. This eliminates double-booking.
- **30-day calibration check** — Full automation of high-confidence cases is not permanent until 30 days of monitoring confirms the error rate stays below 3%.

Why this is different from the two earlier AI projects

- Earlier chatbot: tried to change how hospitals send requests. This project keeps their process the same.
- Earlier recommendation engine: coordinators did not trust it. This project starts with high-confidence cases only and keeps coordinators in control for all borderline decisions.

CEO Feedback — Three Questions Answered

Changes made to the plan after review with Marcus Reyes

Marcus raised three concerns. All three have been addressed.

Q1: The board update is in 6 weeks, not 8. What gets cut?

Done. Phase 1 now delivers the matching agent running on real volume by week 6. The coordinator dashboard UI is moved to weeks 7–8. The agent functionality is unchanged — only the review interface is delayed. Coordinators work from email and SMS alerts during weeks 1–6.

Q2: The 30-day mismatch rate is too slow. I need a signal I can see in week 2.

Done. Two leading signals are now tracked every week from week 2: (1) hospital acceptance rate per confidence band — a drop shows the agent is drifting before any mismatch is reported; (2) coordinator flag rate per week — a rise shows the agent and coordinator judgment are moving apart. The 30-day gate remains but is no longer the only signal.

Q3: Coordinators check the queue every 1–2 hours during busy periods. What happens to a proposal when the 30-minute window expires unreviewed?

Done. The recall window has been extended to 90 minutes — long enough to cover one full batch-check cycle. If the window expires without the coordinator opening the proposal, it escalates to the team lead. It is never automatically approved without a human review.

The Ask

Phase 1 approval to prove the ROI signal

Phase 1 — 6 weeks, measurable result

- **What is built:** Intake parsing agent + matching agent for high and mid-confidence cases
- **What is deferred:** Coordinator dashboard UI (weeks 7–8); Agent 3 confirmation loop (Phase 2)
- **Target:** 50% of daily proposals handled automatically; fill time under 1 hour
- **Board demo:** Matching agent live on real volume at week 6
- **Success signal:** Weekly acceptance rate trend + coordinator flag rate from week 2; mismatch rate tracked separately from hospital preference rejection

What is needed from IT

- ServiceNow API access — read queue, write structured shift record back
- Read access to nurse database — credentials, availability, history
- Confirmation on how current the availability data is — assumed under 24-hour delay
- SMS and email gateway setup for nurse confirmation notifications

Decision needed: approve Phase 1 build to prove ROI before Phase 2 investment.

Phase 2 (weeks 9–16) and Phase 3 (weeks 17–24) are scoped and ready — pending Phase 1 results.